

CHILD WELFARE SYSTEM CONTACT IN THE GLOBAL NORTH: TRENDS FROM 44 COUNTRIES

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ABSTRACT

Background

Child maltreatment and child welfare system contact are both associated with elevated risks of physical and mental morbidity and mortality in childhood, adolescence, and adulthood. Yet existing data on contextual variation in system contact is limited to only a small handful of countries, limiting knowledge in this area.

Methods

We identified and harmonized data on three points of children's contact with the child welfare system: having a child welfare agency investigation for alleged maltreatment; having an agency confirm that maltreatment occurred; and being placed in out-of-home care. For each, we calculated the rate of contact per 1,000 children. Across all outcomes considered, we collected reliable data from 44 countries, a substantial improvement over existing studies focusing on much smaller numbers of countries.

Findings

There is considerable variation in levels of child welfare system contact across countries in the Global North. The incidence of investigations and of confirmed maltreatments has broadly increased in the twenty-first century, while the incidence and prevalence of out-of-home care have remained stable or decreased in the last several years. Cross-national variation in trends is generally less substantial than cross-national variation in levels of child welfare system contact, supporting the idea that countries have stable differences in the incidence/prevalence of system contact. Cross-national variation in out-of-home care reflects, though is not reducible to, regional and sociocultural variation.

Interpretation

Child welfare system contact is very common in many countries in the Global North, but infrequent in several others. Future research should seek to better understand the factors that shape cross-national differences in rates of system contact, with special attention to the degree to which these differences represent variation in underlying rates of child maltreatment or variation in policy, administration, or other factors.

Funding

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RESEARCH IN CONTEXT

Evidence before this study

We searched PubMed using the search terms “child welfare” or “child maltreatment” and “cross-national” or “countries” to identify studies published in English between Jan 1, 2000 and Jun 1, 2022. Previous comparative studies of child maltreatment or child welfare system contact ranged from comparing two countries to eight countries. Overall, these studies revealed that rates of child maltreatment and child welfare system contact varied across the small number of countries for which data were comparable.

Added value of this study

We substantially expanded to 44 the number of countries included in any single comparative study of child maltreatment or child welfare system contact. Comparing the child welfare landscapes in a larger number of countries allows child welfare researchers, practitioners, and policymakers to better contextualize their work. We also created the publicly available ROCKWOOL-Duke Global Child Welfare Database, which can serve as a resource for the child health and welfare research community by centralizing data on child maltreatment and child welfare system contact that are publicly available but currently dispersed across different sources.

Implications of all the available evidence

Our study, combined with past comparative child maltreatment studies, reveals that rates of contact with child welfare systems occur at non-negligible frequencies across national contexts. Specific countries consistently suffer from higher rates of child maltreatment and manage higher rates of contact with the child welfare system than others; thus, future research, policy, and practice should focus on the implications of these phenomena with regards to population health and cross-national disparities in population health. Additional research is needed to unearth potential political, social, cultural, and economic mechanisms explaining cross-national variation in child maltreatment and contact with the child welfare system such that knowledge can be shared across borders to improve outcomes for all.

INTRODUCTION

Child maltreatment and child welfare system (CWS) or child protective services (CPS) involvement (we use the term CWS/CPS throughout) account for significant health and economic burdens in the Global North.^{1–6} Children who have experienced maltreatment or who have been exposed to CWS/CPS are significantly and often quite substantially more likely to have poor health and wellbeing across the life-course, including elevated risks of mortality, worse educational outcomes, higher risk of having a child as a teenager, higher rates of contact with the criminal justice system, and worse physical and mental health outcomes than similar children who had not experienced these events.^{7–14}

To the degree such evidence exists, evidence indicates that the effects of exposure to maltreatment and to CWS/CPS are relatively consistent across national and subnational contexts.^{1–15} But dramatically less is known about how much rates of exposure to maltreatment and system contact vary across national and historical contexts.^{16–18} This lack of evidence, which is driven partially by contextual differences in the definition of child maltreatment and partially by differences in how CWS/CPS responds to allegations of maltreatment, are pressing for the medical and public health communities for three reasons.¹⁹ First, and perhaps most importantly, they are relevant because both maltreatment and exposure to CWS/CPS are salient risk factors for poor mental and physical health for children, making our lack of knowledge about their relative levels an important gap in research. Second, both of these events are highly unequally distributed by race/ethnicity and social class within countries,^{20–26} meaning that they have the capacity to exacerbate health inequities and, in a similar vein, to dampen health disparities if minimized (in the case of maltreatment) or handled in health-promoting ways (in the case of system contact). Third, they display core differences in how states respond to child maltreatment, with likely implications for variations in population health across national and subnational contexts.

In this article, we extend earlier, seminal work by Gilbert and colleagues¹⁶ by providing the most extensive information to date on how rates of contact with CWS/CPS—specifically, investigation rates, confirmed maltreatment rates, and out-of-home placement rates—vary across countries in the Global North. We focus on these three different levels of CWS/CPS contact because each of these three stages of CWS/CPS contact is a consequential intervention into family life and because capturing actual levels of maltreatment cross-nationally is virtually impossible on account of differing definitions of maltreatment and differential measurement error across contexts. In so doing, we expand our knowledge about how common three state interventions in response to child maltreatment are and, as a result, how significant a role each may play in shaping population health and disparities therein. Further, we also introduce the new publicly available ROCKWOOL-Duke Global Child Welfare Database, which can serve as a future resource for the child health and welfare research community.

METHODS

This cross-sectional study used secondary data files, so we did not seek institutional review board (IRB) approval. The study follows the Strengthening the Reporting of Observational Studies in Epidemiology (STROBE) reporting guideline for observational studies.

Measures

Our outcomes were three types of contact with the child welfare system (CWS) or child protective services (CPS). Investigation was defined as an official inquiry by CWS/CPS personnel resulting from an allegation of children maltreatment. Investigations were counted multiple times if children were re-investigated within a twelve-month period or if investigations involved multiple children. Countries varied in whether they reported investigations started or completed in a twelve-month period.

Confirmed maltreatment was defined as an investigation resulting in the substantiation of alleged maltreatment. In the United States, indications of maltreatment were treated as confirmed maltreatment. Confirmed maltreatment in Canada. And Germany included the substantiation of risk for future maltreatment.

Out-of-home care was defined as living without parents under the auspices of a CWS/CPS agency. As a result, the definition included foster care, kinship care, and various institutional care settings. We excluded children residing in criminal justice institutions, but because child behavioral issues cannot be easily separated from child maltreatment, we included children in out-of-home care administered by CWS/CPS even if the primary reason for placement was child behavioral issues.

For investigations and confirmed maltreatments we calculated the annual incidence (investigations/confirmed maltreatments per twelve-month period per 1,000 children) and the annual prevalence (distinct children experiencing investigation/confirmed maltreatment per twelve-month period per 1,000 children). The main analysis focuses on the incidence of investigation/maltreatment, but our findings are broadly consistent with analysis of the prevalence of investigation/confirmed maltreatment (figures S1 and S2). We calculated the annual incidence of placement into out-of-home care (entrances into care per twelve-month period per 1,000 children) and the point prevalence of out-of-home care (children in care per 1,000 children). Countries varied in the dates on which point-prevalence was measured.

Our exposures were country and year. For analysis of out-of-home care, we also organized countries into geographic and sociocultural groups using United Nations geographic regions and a group for Anglophone countries of the Global North.

Data collection

Data collection occurred between August 2021 and May 2022. We sought data from 64 countries in the Global North for the years 2000–2020 (table S1). For simplicity, we refer to Greenland and the constituent countries of the United Kingdom (England, Northern Ireland, Scotland, and Wales) as countries.

Our principal sources of data were tabulated data files and statistical reports published by national statistical agencies and national child welfare agencies, accessed through agency websites or national databases (table S2). If data were not publicly available, we accessed data directly from relevant agencies. If no data could be accessed through national agencies, data from UNICEF's TransMonEE database and the Nordic Health and Welfare Statistics database were used. When multiple sources of data were discrepant, we investigated further to determine the correct figure, or removed data when

a reliable figure could not be established. Agency personnel and subject-area experts provided support accessing, translating, and interpreting data.

With one exception, we limited data collection to administrative data on children aged 0–17. For Canada we used administrative survey data on children aged 0–15. Comparing data from Canada to peer countries indicates that any bias resulting from differing age ranges is small (figure S3).

We obtained data from 44 countries, including 104 country–year observations of rates of CWS/CPS investigations, 194 country–year observations of rates of CWS/CPS confirmations of maltreatment, 288 country–year observations of rates of children entering out-of-home care, and 402 country–year observations of children residing in out-of-home care at a point in time (figure 1).

[Figure 1 about here.]

Data analysis

Data analysis was conducted using R statistical software version 4.2.0 (R Project for Statistical Computing) between February 2022 and June 2022.

Data and materials availability

The ROCKWOOL–Duke Global Child Welfare Database will be freely available through the National Data Archive on Child Abuse and Neglect, which is a collaboration between Cornell University and Duke University. Because the data are aggregate in nature and do not pose any disclosure risk for individual children exposed to CWS/CPS, they are available without IRB approval. Code for replicating the analysis is available on the Open Science Framework (<https://osf.io/wfqpb/>).

Role of the funder

The funders of the study had no role in study design, data collection, data analysis, data interpretation, or writing or editing of this article. All authors had full access to all of the data published in the study, and the corresponding author had final responsibility for the decision to submit the manuscript.

RESULTS

Investigation

Across 12 countries for which reliable data were available on CWS/CPS investigations for alleged maltreatment, incidence rates varied widely (figure 2A). In recent years, the United States had the highest rates of investigation, peaking at 58·37 investigations per 1,000 children in 2017. England had comparably high rates of investigation. In contrast, Singapore had extremely low rates of investigation, ranging from 0·16 to 1·51 investigations per 1,000 children between 2002 and 2020. As a result, observed rates of investigation in the Global North varied by a factor of 45·6 in 2019.

[Figure 2 about here.]

Levels of investigation increased in most countries over the period for which data was obtained (figure 2A). Rates of investigation increased steadily and substantially in the United States and

Germany. Increases in rates of investigation accelerated in Japan between 2010 and 2019, accounting for a more than threefold increase over the decade. Uniquely, New Zealand experienced a marked decline in investigations in the mid-2010s. Data for Canada and Ireland were inadequate for evaluating trends.

Confirmed maltreatment

Data on incidents of maltreatment confirmed by CWS/CPS were gathered for 19 countries (figure 2B). Cross-national variation in rates of confirmed maltreatment was considerably less than cross-national variation in rates of investigation (figure 2A), with the majority of countries having rates of confirmed maltreatment in the range of 2.5 to 6.0 maltreatments per 1,000 children in recent years. Nevertheless, countries like Canada and New Zealand displayed very high (if decreasing) rates of confirmed maltreatment, while countries like Bulgaria and Tajikistan exhibited extremely low rates of confirmed maltreatment. South Korea had very low rates of confirmed maltreatment early in the twenty-first century, before converging with most other countries in recent years.

Rates of confirmed maltreatment had steady, meaningful, and broad-based increases over the twenty-first century (figure 2B). There were, however, important exceptions to this trend. In 2013–2015 New Zealand experienced a sharp decline in its rate of confirmed maltreatment that coincided with the decline in its rate of investigation (figure 2A). Rates of confirmed maltreatment decreased in the United States between 2005 and 2010, remaining fairly stable thereafter. Australia experienced a dramatic drop in confirmed maltreatments in 2018, but rates of maltreatment returned to prior levels in 2019–2020.

Out-of-home care

Of the three points of child welfare system contact analyzed in this study, reliable data on out-of-home care was most widely available. Across fully 25 countries within the Global North, annual rates of children's entrance into out-of-home care varied significantly (figure 3A), although nearly half of countries observed in 2020 had entrance rates between 2.0 and 3.0 entrances per 1,000 children. In recent years, Sweden had the highest rate of entrance into out-of-home care, at 4.05 entrances per 1,000 children in 2020. (Data for Sweden in 2014–2017 were not comparable due to administrative changes.) Albania had the lowest recorded rate of out-of-home care entrance, at 0.22 per 1,000 children in 2020. As a result, observed rates of foster care entry in the Global North varied by a factor of 18.3 in 2019.

[Figure 3 about here.]

Most countries exhibited fairly stable rates of entry into out-of-home care. Two major exceptions were Sweden and Germany, which experienced dramatic peaks in entrances in the mid-2010s. Based on limited data on the migrant status of children entering care, a substantial proportion of these increases appear to be attributable to large, temporary increases in the number of unaccompanied children who entered these countries and were placed into care (figure S4). Though varying in their timing, Russia, Latvia, Lithuania, and Estonia all experienced significant declines from high rates of entry into care. A larger number of countries experienced more modest declines in entry rates between 2015 and 2020.

Data on children residing in out-of-home care on a specific date was available for 37 countries (figure 3B). Although data was only available for two years, rates of children's residence in out-of-home care in Greenland were grossly outlying, at 67·88 children in care per 1,000 children in 2006—or more than 1 in 15 children—and 53·91 children in care per 1,000 children in 2019—more than 1 in 19 children. In these years, rates of out-of-home care in Greenland were more than 2·5 and 3·3 times greater, respectively, than the next highest national rate of care.

Excluding Greenland from visualization helps interpret variation in out-of-home care among other countries (figure 3C). A majority of countries observed in 2020 had care rates between 5·0 and 10·0 children in care per 1,000 children, or between 0·5 and 1·0 percent of the child population. Russia exhibited the highest rates of out-of-home care, peaking at 28·42 in 2008, but data were unavailable after 2015. Comparing extrema in 2020—16·45 children in care per 1,000 children in Latvia and 1·27 children in care per 1,000 children in Singapore—rates of children in out-of-home care varied by a factor of nearly 13. Reflecting similar trends in entrances into out-of-home care (figure 3A), in recent years the point prevalence of residence in out-of-home care in many countries either ceased to grow or experienced modest declines.

Cross-national variation in rates of out-of-home care was partly a reflection of variation in care rates across geographic and cultural regions. Observed countries in Western, Central, Eastern, and South-eastern Asia (Armenia, Azerbaijan, Georgia, Japan, Kazakhstan, Singapore, South Korea, Turkey, and Uzbekistan) exhibited many of the lowest rates of intake into care (figure 4A) and shares of children in care (figure 4B). Though geographically diverse, Anglophone countries (Australia, Canada, England, Ireland, New Zealand, Northern Ireland, United States, Scotland, and Wales) displayed very similar levels and trends in out-of-home care.

[Figure 4 about here.]

Rates of out-of-home care varied more widely within the European context. Eastern European countries (Belarus, Bulgaria, Czech Republic, Hungary, Poland, Moldova, Russia, and Slovakia) generally had much higher rates of care than Southern European countries (Albania, Croatia, Montenegro, and Spain). Reliable data for Western European countries was only obtained for Germany, which had relatively high rates of entrance into care, and for France, which had rates of residence in care that fell in the middle of the range of other European countries.

Among Northern European countries for which data were obtained, rates of entrance into out-of-home care were consistently among the highest observed in the Global North (figure 4A). However, the share of children in out-of-home care varied more widely among observed Northern European countries (figure 4B), with countries like Latvia and Lithuania having among the highest rates of residence in care, and countries like Estonia and Iceland having among the lowest rates of residence in care.

DISCUSSION

Our results demonstrate significant cross-national variation in child welfare system and child protective services (CWS/CPS) contact across the Global North. But they also show that rates of investigation, confirmed maltreatment, and out-of-home care are in most cases fairly stable, and that many trends in system contact are shared across many countries. Furthermore, countries within geographic regions and countries sharing social and legal institutions exhibit similarities in system

contact. These facts support the conclusion that large-scale comparative research on child welfare system contact is both feasible and likely to be fruitful.

A key question that arises from our findings is the extent to which cross-national variation in child welfare system contact results from differences across countries in their underlying incidence of child abuse and neglect or in their child welfare policies and administrative personnel and procedures. Further research is needed to understand the relative importance of behavioral and institutional factors. As with virtually all research in this area, this represents a core challenge for researchers who are interested in child maltreatment and CWS/CPS contact. Because harmonized data on CWS/CPS contact is now broadly available through the ROCKWOOL-Duke Global Child Welfare Database—introduced in this study—there are now valuable resources for understanding the various national factors that contribute to both levels and changes in levels of system contact. Yet because no harmonized national data on actual maltreatment exposure exist, it remains difficult, if not impossible, to decipher the degree to which rates of underlying maltreatment mediate the relationship between other national factors—the generosity of welfare regimes,^{27,28} the level of income inequality,²⁹ or the state of the criminal justice system,³⁰ to name just three—and rates of CWS/CPS contact.

Our data are not adequate to directly evaluate qualitative differences in CWS/CPS across countries. But several comparisons offer suggestive evidence about qualitative variation. First, rates of investigation, confirmed maltreatment, and out-of-home care are not always consistent within countries: some countries display relatively high rates of some measures and relatively low rates of others. This suggests that countries vary in their transition probabilities from investigation to confirmation to out-of-home placement. Second, the incidence of entry into care and the point prevalence of care also diverge in several countries, most notably Sweden, which had among the highest rates of entrance into care but a relatively low rate of residence in care. This indicates that countries vary meaningfully in the duration that children spend in out-of-home care, another core area meriting future research using these data. Taken together, these two points of evidence suggest that countries in the Global North may differ meaningfully in the ways that they handle child maltreatment cases, conditional on children's initial contact with CWS/CPS.

Despite the value added by our new, large-scale dataset, systematic inferences cannot be made from our sample. We sought data from 64 countries and obtained data on at least one measure from 44 countries. Our inability to access data may be systematically related to levels of child maltreatment or CWS/CPS contact. In this case our sample may lead to biased inferences about the Global North. Countries in the Global North are characterized by a wide range of per-capita income levels and levels of social support, and so general conclusions cannot be drawn about high-income countries or countries with well-developed welfare states. Whereas it is possible to compare several countries across indicators, there are still many countries in the Global North where there is a dearth of publicly available knowledge about children's contact with CWS/CPS. Given the high individual and social cost of child maltreatment, it is paramount that countries secure and make available data on children's contact with CWS/CPS.

DECLARATION OF INTEREST

The authors declare no conflict of interest.

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Figure 1: Range of data on (A) child protective services investigations, (B) cases of confirmed maltreatment, and (C) children in out-of-home care for 44 countries in the Global North, 2000-2020.

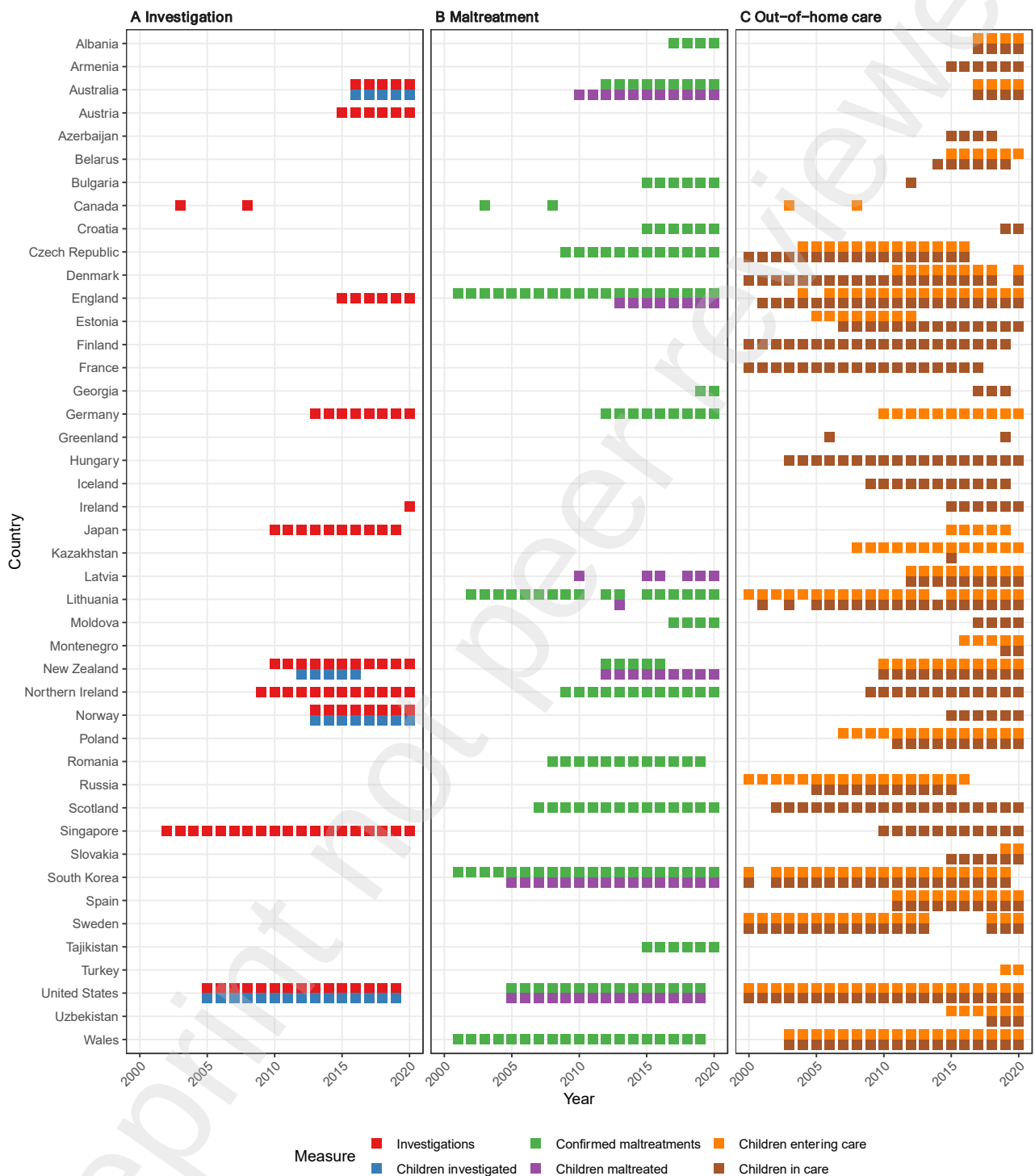


Figure 2: Annual incidence of (A) child protective services investigations and (B) maltreatments confirmed by child protective services.

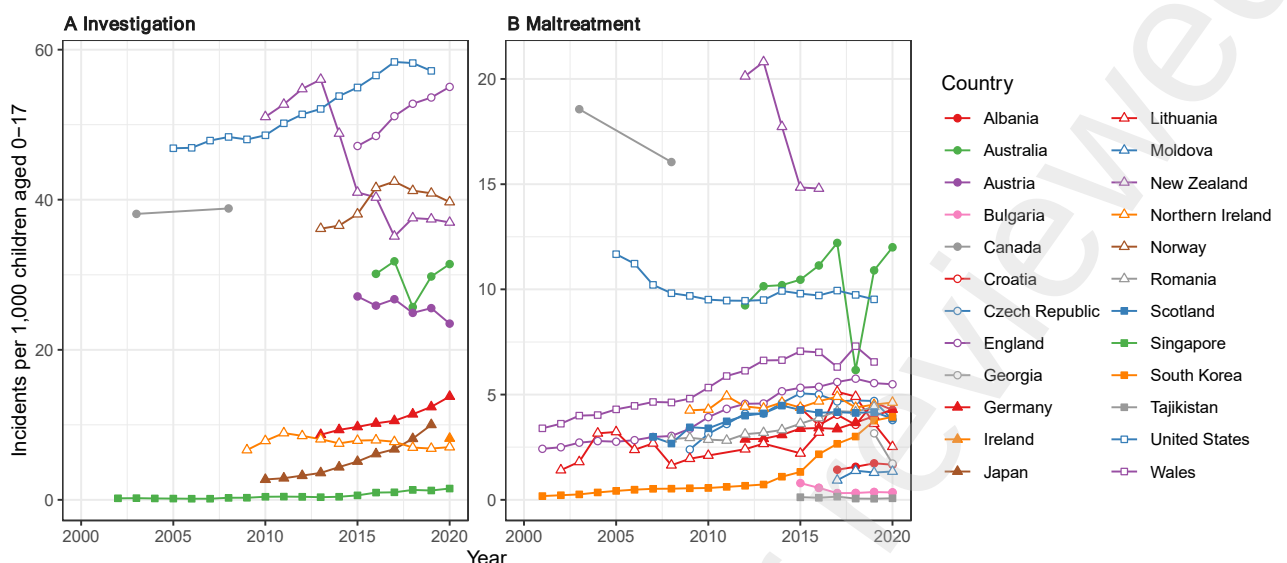


Figure 3: Annual incidence of (A) children's entrance into out-of-home care and point prevalence of (B, C) children's residence in out-of-home care. The detail panel (C) excludes Greenland. Point prevalence is measured at different dates in different countries.

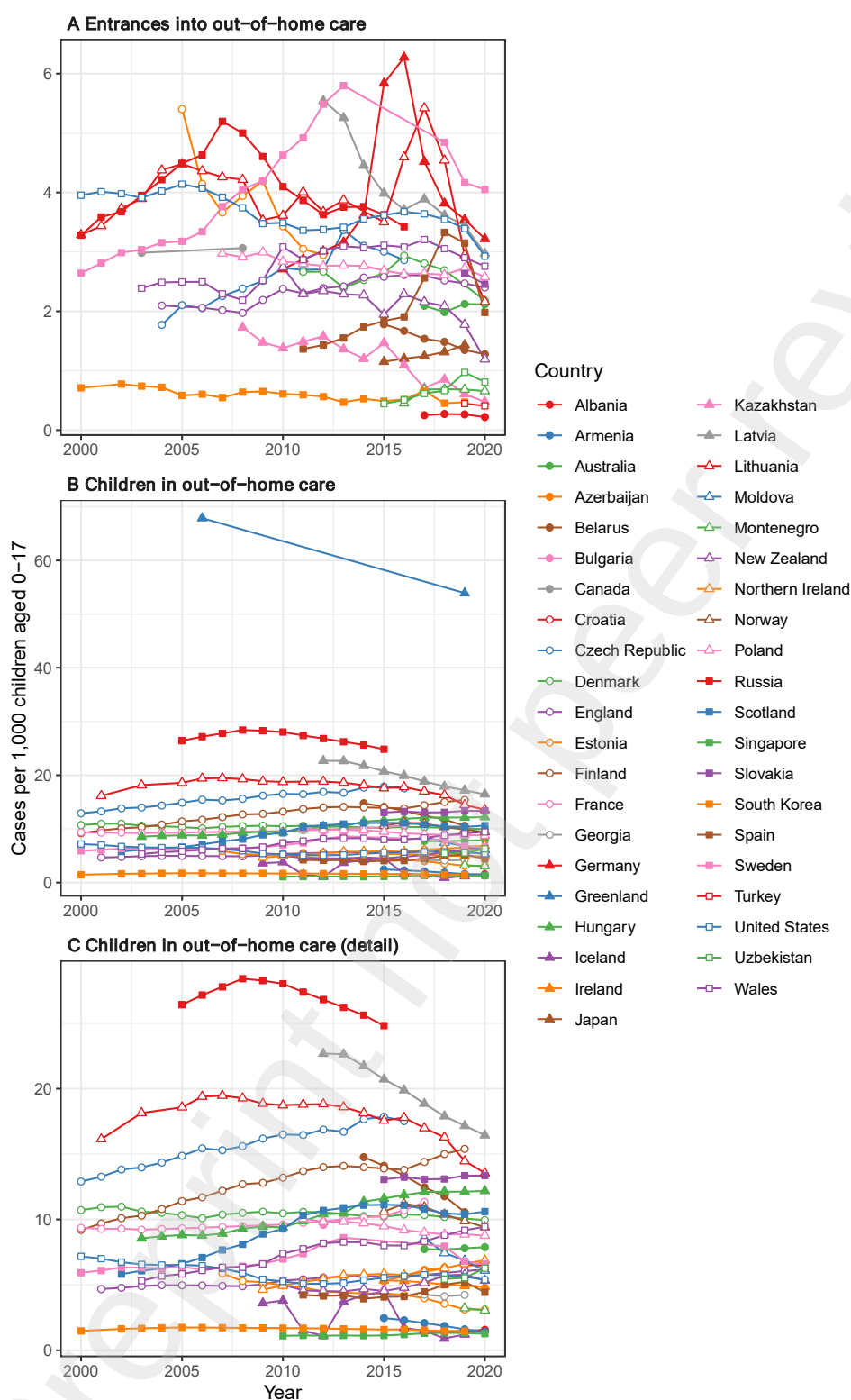


Figure 4: Incidence of (A) children's entrance into out-of-home care and point prevalence of (B) children's residence in out-of-home care in four regions of the Global North. For visualization, the figure excludes Greenland.

